

Draft - expert review pending

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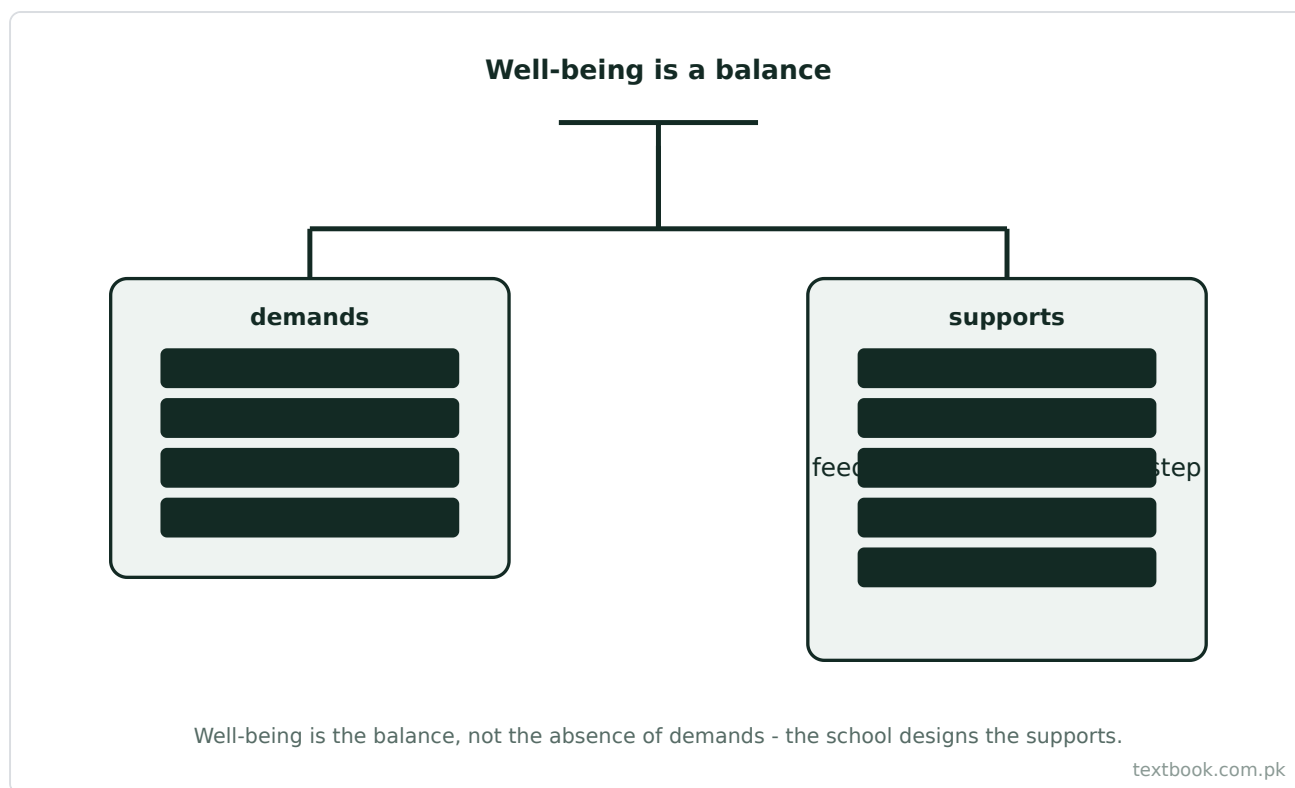
Draft translation - not yet human-reviewed / مسوده ترجمه

Topic 11.1 - What mental health and well-being are, and how they show in school

A real classroom situation

Two Class 8 pupils in a Karachi school. One has high marks and constant dread: she revises until midnight, cries over a dropped mark, and cannot say why. The other has middling marks and steady energy: he works, plays, argues about cricket, and sleeps. The staff room has one word for the first - "brilliant" - and no word for the second at all.

The unit's first claim is that the staff room has the well-being of these two pupils exactly backwards. Well-being is not the marks; it is the balance a pupil's life holds. And the second claim follows: a school that cannot tell the difference will miss the pupil who needs it most - because the loudest distress often arrives wearing a tie.



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Explanation

What mental health and well-being mean

The World Health Organization defines mental health as "a state of mental well-being that enables people to cope with the stresses of life", to realise their abilities, to learn and work well, and to contribute to their community - adding that it "has intrinsic and instrumental value and is a basic human right" (World Health Organization, 2026). Three consequences follow for a teacher.

Mental health is more than the absence of illness. The definition is a state of functioning, not a diagnosis. A pupil can be free of any condition and still be unwell - the high marks and constant dread - and a pupil with a diagnosed condition can be functioning well with the right supports. The school's question is not "who is ill?" but "who is coping, and who is not?"

Well-being is a balance, not a feeling. The most useful classroom image is a balance of **demands** against **supports**. Demands: the examination schedule, the comparison culture, the noisy hot room, the load some pupils carry at home. Supports: routine, belonging, feedback that names the next step, safety, and

the one adult who notices. Well-being is not the absence of demands - school IS demands

- but the presence of supports that meet them. The balance explains the unit's opening scene: the brilliant pupil's pan is loaded on one side only.

Coping can be learned. WHO's framing of coping as part of the state means well-being is partly a skill set, not a fixed endowment - which is why the school's supports work, and why "school-based social and emotional learning programmes are especially effective across all income levels" (World Health Organization, 2026). The school is not a bystander to well-being; it is one of its supports.

The misconception to clear now. "Mental health means mental illness, and it is not a school's business." The first half mistakes a state for a diagnosis; the second mistakes the school for a bystander. The pupil who cannot learn while afraid has been this course's running example since Unit 2 - well-being is the condition every other unit has been quietly assuming. What is NOT the school's business - diagnosis, therapy, treatment - is exactly what this unit's referral line is for.

Common difficulties and their signs in school

The difficulties a Sindh classroom most often meets: **anxiety** (exam nerves that have outgrown the exam), **persistent low mood** (the withdrawal that lasts weeks, not days), **conduct difficulties** (the behaviour that carries a message), and - the one this topic names with care - **hopelessness**, the talk that treats the future as closed.

The signs a teacher can notice are observable, and none of them is a diagnosis: withdrawal from friends and activities; a fall in marks that has no teaching explanation; irritability or anger out of scale with its trigger; absence patterns that cluster (Mondays, after weekends); changes in appearance or self-care; and talk of hopelessness, of being a burden, of the future as closed. Two signs demand same-day escalation regardless of anything else: talk of self-harm, and a sudden calm in a pupil who has been in visible distress - the second because it can mean a decision has been made.

The teacher's discipline with these signs is threefold. **Notice without diagnosing**: the sign is a reason to look and to listen, never a label to apply. **Read the pattern, not the day**: every pupil has a bad Monday; the sign that matters is the change that persists. **Treat the talk as real**: a pupil's hopeless sentence is the most reliable evidence available, and dismissing it as drama is the costliest move in the school's repertoire.

sign a teacher can notice	a possible meaning	the response within the teacher's role
withdrawal from friends and activities	isolation, low mood, fear of the room	a private, calm check-in; keep the pupil included by role
a fall in marks with no teaching explanation	something outside the lesson has changed	look wider before re-teaching; ask, do not assume
irritability out of scale with its trigger	the load has exceeded the supports	reduce the demand; name the feeling calmly
absence that clusters (Mondays, after weekends)	the home demand is heavy	a quiet conversation, not a public question
talk of hopelessness	the pupil is telling you the most important thing they can	listen fully, never dismiss, escalate the same day

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Activity: The balance of one pupil

In pairs, about 20 minutes.

Take one anonymised pupil you both know something about - from practicum, from schooling, from family - whose year worried you.

1. List the demands on the pupil's left pan: school, home, comparison, anything you know.
2. List the supports on the right pan: what met those demands, and what did not.
3. Place the signs you saw on the table: which signs, how long, in what pattern?

4. Then the discipline question: which of your readings is evidence, and which is assumption? Mark every assumption as one.

Check your understanding

1. State WHO's definition of mental health in one sentence, and name what it is more than. *(Remember)*
2. Explain the demands-and-supports balance with the two Class 8 pupils. *(Apply)*
3. Why is "who is ill?" the wrong school question? Give the right one. *(Understand)*
4. Name any four signs a teacher can notice, and the rule for reading them. *(Remember)*
5. Which two signs demand same-day escalation, and why the second one? *(Remember)*

Summary

Mental health, in WHO's framing, is a state of well-being that enables coping, realising abilities and contributing - more than the absence of illness, and a basic human right. Well-being is a balance of demands against supports, and the school is one of the supports: its routines, belonging, feedback and safety are well-being machinery, and social-emotional learning works across income levels. The difficulties a classroom meets show in observable signs - withdrawal, unexplained falls, out-of-scale irritability, clustered absence, and the talk of hopelessness that must always be taken as real. The teacher's discipline: notice without diagnosing, read the pattern not the day, and escalate same-day for self-harm talk or a sudden calm in distress.

Self-assessment checklist

Sign in to sync your ticks across devices.

- ☐ I can state WHO's definition and its three consequences for a teacher.
- ☐ I can draw the demands-and-supports balance for a real pupil.

- ☐ I can name the observable signs and the rule for reading them.
- ☐ I know the two same-day escalation signs and why each one.

Try this at your practicum school

For one week, keep a private tally of the school day's demands and supports as your class experiences them: every comparison made public, every routine that held, every wrong answer made safe. The tally is the school's well-being balance, seen from the pupils' side.

Summative task

Write **250-300 words** on the brilliant pupil.

The high marks and constant dread have been visible since September. Using this topic, analyse her well-being balance (demands, supports, what the staff room sees and does not), name the signs with their pattern, and describe the response within the teacher's role - what you would do this week, and what you would NOT do.

Mini-rubric

Criterion	What a strong answer shows
The balance is drawn honestly	Both pans named with the evidence available.
The signs are read as a pattern	Duration and clustering, not a single bad day. (<i>Analyze</i>)
The role limit is exact	What the teacher does, and where the referral line sits.
Plain, accurate writing	Terms used correctly; no padding; within the word count.

Further reading

- World Health Organization. (2026). *Mental health: Strengthening our response* (fact sheet). <https://www.who.int/news-room/fact-sheets/detail/mental-health-strengthening-our-response>
- Seifert, K., & Sutton, R. (2009). *Educational Psychology* (2nd ed.), Chapter 8. University of Manitoba. CC BY. <https://open.umn.edu/opentextbooks/textbooks/educational-psychology>

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